



CEROBACT 1.2g

(Amoxicillin 1000mg and Clavulanic Acid 200mg Powder for solution for infusion)

Please read the package insert carefully and use under physician's instruction

[Active Ingredient]

Each vial contains amoxicillin sodium equivalent to 1000 mg amoxicillin and potassium clavulanate equivalent to 200 mg of clavulanic acid.

[Dosage Form]

Powder for Solution for Infusion

[Product Description]

It is a white or off-white powder.

Reconstituted solutions are normally colourless or a yellow colour.

Diluted solutions for intravenous infusion are normally colourless or a slightly yellow colour.

No diluent is provided together with the product for reconstitution.

Diluent(s) to be used for reconstitution and dilution of this product are Water for Injection, Sodium Chloride Injection 0.9%, Sodium Lactate Ringer's Injection.

[Pharmacodynamics]

Pharmacotherapeutic group: Combinations of penicillins, incl. beta-lactamase inhibitors; ATC code: J01CR02.

Mechanism of action

Amoxicillin is a semisynthetic penicillin (beta-lactam antibiotic) that inhibits one or more enzymes (often referred to as penicillin-binding proteins, PBPs) in the biosynthetic pathway of bacterial peptidoglycan, which is an integral structural component of the bacterial cell wall. Inhibition of peptidoglycan synthesis leads to weakening of the cell wall, which is usually followed by cell lysis and death.

Amoxicillin is susceptible to degradation by beta-lactamases produced by resistant bacteria and therefore the spectrum of activity of amoxicillin alone does not include organisms which produce these enzymes.

Clavulanic acid is a beta-lactam structurally related to penicillins. It inactivates some beta-lactamase enzymes thereby preventing inactivation of amoxicillin. Clavulanic acid alone does not exert a clinically useful antibacterial effect.

Pharmacokinetic/pharmacodynamic relationship

The time above the minimum inhibitory concentration (T>MIC) is considered to be the major determinant of efficacy for amoxicillin.

Mechanisms of resistance

The two main mechanisms of resistance to amoxicillin/clavulanic acid are:

• Inactivation by those bacterial beta-lactamases that are not themselves inhibited by clavulanic acid, including class B, C, and D.

• Alteration of PBPs, which reduce the affinity of the antibacterial agent for the target.

Impermeability of bacteria or efflux pump mechanisms may cause or contribute to bacterial resistance, particularly in Gram-negative bacteria.

Breakpoints

MIC breakpoints for amoxicillin/clavulanic acid are those of the European Committee on Antimicrobial Susceptibility Testing (EUCAST)

Organism	Susceptibility Breakpoints (µg/ml)		
	Susceptible	Intermediate	Resistant
<i>Haemophilus influenzae</i> ¹	≤ 1	-	> 1
<i>Moraxella catarrhalis</i> ¹	≤ 1	-	> 1
<i>Staphylococcus aureus</i> ²	≤ 2	-	> 2
<i>Coagulase-negative staphylococci</i> ²	≤ 0.25	-	> 0.25
<i>Enterococcus</i> ³	≤ 4	8	> 8
<i>Streptococcus A, B, C, G</i> ³	≤ 0.25	-	> 0.25
<i>Streptococcus pneumoniae</i> ³	≤ 0.5	1-2	> 2
<i>Enterobacteriaceae</i> ⁴	-	-	> 8
<i>Gram-negative Anaerobes</i> ¹	≤ 4	8	> 8
<i>Gram-positive Anaerobes</i> ¹	≤ 4	8	> 8
<i>Non-species related breakpoints</i> ¹	≤ 2	4-8	> 8

¹ The reported values are for amoxicillin concentrations. For susceptibility testing purposes, the concentration of clavulanic acid is fixed at 2 mg/l.

² The reported values are oxacillin concentrations.

³ Breakpoint values in the table are based on ampicillin breakpoints.

⁴ The resistant breakpoint of R>8 mg/l ensures that all isolates with resistance mechanisms are reported resistant.

⁵ Breakpoint values in the table are based on benzylpenicillin breakpoints.

The prevalence of resistance may vary geographically and with time for selected species, and local information on resistance is desirable, particularly when treating severe infections. As necessary, expert advice should be sought when the local prevalence of resistance is such that the utility of the agent in at least some types of infections is questionable.

Commonly susceptible species	Species for which acquired resistance may be a problem
Aerobic Gram-positive micro-organisms	Aerobic Gram-positive micro-organisms
<i>Enterococcus faecalis</i>	<i>Enterococcus faecium</i> §
<i>Gardnerella vaginalis</i>	Aerobic Gram-negative micro-organisms
<i>Staphylococcus aureus</i> (methicillin-susceptible) ‡	<i>Escherichia coli</i>
<i>Coagulase-negative staphylococci</i> (methicillin-susceptible)	<i>Klebsiella oxytoca</i>
<i>Streptococcus agalactiae</i>	<i>Klebsiella pneumoniae</i>
<i>Streptococcus pneumoniae</i> ¹	<i>Proteus mirabilis</i>
<i>Streptococcus pyogenes</i> and other beta-haemolytic streptococci	<i>Proteus vulgaris</i>
<i>Streptococcus viridans</i> group	Inherently resistant organisms
Aerobic Gram-negative micro-organisms	Aerobic Gram-negative micro-organisms
<i>Acinetobacter baumannii</i>	<i>Acinetobacter</i> sp.
<i>Citrobacter freundii</i>	<i>Citrobacter</i> sp.
<i>Enterobacter</i> sp.	<i>Legionella pneumophila</i>
<i>Legionella pneumophila</i>	<i>Morganella morganii</i>
<i>Morganella morganii</i>	<i>Providencia</i> sp.
<i>Providencia</i> sp.	<i>Pseudomonas</i> sp.
<i>Pseudomonas</i> sp.	<i>Serratia</i> sp.
<i>Serratia</i> sp.	<i>Stenotrophomonas maltophilia</i>
<i>Stenotrophomonas maltophilia</i>	Other micro-organisms
Other micro-organisms	<i>Chlamydia trachomatis</i>
<i>Chlamydia trachomatis</i>	<i>Chlamydophila pneumoniae</i> ¹
<i>Chlamydophila pneumoniae</i> ¹	<i>Chlamydophila psittaci</i>
<i>Chlamydophila psittaci</i>	<i>Coxiella burnetii</i>
<i>Coxiella burnetii</i>	<i>Mycoplasma pneumoniae</i>
<i>Mycoplasma pneumoniae</i>	

§ Natural intermediate susceptibility in the absence of acquired mechanism of resistance.

‡ All methicillin-resistant staphylococci are resistant to amoxicillin/clavulanic acid.

§ All strains with resistance to amoxicillin that is not mediated by beta-lactamases are resistant to amoxicillin/clavulanic acid.

¹ This presentation of amoxicillin/clavulanic acid may not be suitable for treatment of *Streptococcus pneumoniae* that are resistant to penicillin.

² Strains with decreased susceptibility have been reported in some countries in the EU with a frequency higher than 10%.

[Pharmacokinetics]

Absorption

The pharmacokinetic data of 1000 mg/200 mg given as a bolus intravenous injection are presented below.

Mean pharmacokinetic parameters

Bolus intravenous injection

Dose administered	Dose	Mean peak serum conc (µg/ml)	T 1/2 (h)	AUC (h·mg/l)	Urinary recovery (% 0 to 6 h)
Amoxicillin					
AMX/CA 1000 mg/200 mg	1000 mg	105.4	0.9	76.3	77.4
Clavulanic acid					
AMX/CA 1000 mg/200 mg	200 mg	28.5	0.9	27.9	63.8

AMX – amoxicillin, CA – clavulanic acid

Distribution

About 25% of total plasma clavulanic acid and 18% of total plasma amoxicillin is bound to protein. The apparent volume of distribution is around 0.3-0.4 l/kg for amoxicillin and around 0.2 l/kg for clavulanic acid.

Following intravenous administration, both amoxicillin and clavulanic acid have been found in gall bladder, abdominal tissue, skin, fat, muscle tissues, synovial and peritoneal fluids, bile and pus. Amoxicillin does not adequately distribute into the cerebrospinal fluid.

From animal data there is no evidence for significant tissue retention of drug-derived material for either component. Amoxicillin, like most penicillins, can be detected in breast milk. Trace quantities of clavulanic acid can also be detected in breast milk.

Both amoxicillin and clavulanic acid have been shown to cross the placental barrier.

Biotransformation

Amoxicillin is partly excreted in the urine as the inactive penicilloic acid in quantities equivalent to up to 10 to 25% of the initial dose. Clavulanic acid is extensively metabolized in man, and eliminated in urine and faeces, and as carbon dioxide in expired air.

Elimination

The major route of elimination for amoxicillin is via the kidney, whereas for clavulanic acid it is by both renal and non-renal mechanisms.

Amoxicillin/clavulanic acid has a mean elimination half-life of approximately one hour and a mean total clearance of approximately 25 l/h. Approximately 60 to 70% of the amoxicillin and approximately 40 to 65% of the clavulanic acid are excreted unchanged in urine during the first 6 h after administration of a single 500/100 mg or a single 1000/200 mg bolus intravenous injection. It is found that the urinary excretion to be 50-85% for amoxicillin and between 27-60% for clavulanic acid over a 24 hour period. In the case of clavulanic acid, the largest amount of drug is excreted during the first 2 hours after administration.

Concomitant use of probenecid delays amoxicillin excretion but does not delay renal excretion of clavulanic acid.

Age

The elimination half-life of amoxicillin is similar for children aged around 3 months to 2 years and older children and adults. For very young children (including preterm newborns) in the first week of life the interval of administration should not exceed twice daily administration due to immaturity of the renal pathway of elimination. Because elderly patients are more likely to have decreased renal function, care should be taken in dose selection, and it may be useful to monitor renal function.

Renal impairment

The total serum clearance of amoxicillin/clavulanic acid decreases proportionately with decreasing renal function. The reduction in drug clearance is more pronounced for amoxicillin than for clavulanic acid, as a higher proportion of amoxicillin is excreted via the renal route. Doses in renal impairment must therefore prevent undue accumulation of amoxicillin while maintaining adequate levels of clavulanic acid.

Hepatic impairment

Hepatically impaired patients should be dosed with caution and hepatic function monitored at regular intervals.

[Indication]

CEROBACT should be used in accordance with local official antibiotic-prescribing guidelines and local susceptibility data.

CEROBACT is indicated for short-term treatment of bacterial infections at the following sites:

Upper respiratory tract infections (including ENT) e.g. recurrent tonsillitis, sinusitis, otitis media. Lower respiratory tract infections e.g. acute exacerbation of chronic bronchitis, lobar and bronchopneumonia.

Genito-urinary tract infections e.g. cystitis, urethritis, pyelonephritis.

Skin and soft tissue infections, e.g. boils, abscesses, cellulitis, wound infections.

Bone and joint infections e.g. osteomyelitis.

Other infections e.g. intra-abdominal sepsis.

CEROBACT intravenous is also indicated for prophylaxis against infection which may be associated with major surgical procedures such as gastrointestinal, pelvic, head and neck, cardiac, renal, joint replacement and biliary tract.

Susceptibility to CEROBACT will vary with geography and time (see Pharmacodynamics for further information). Local susceptibility data should be consulted where available, and microbiological sampling and susceptibility testing performed where necessary.

Infections caused by amoxicillin-susceptible organisms are amenable to CEROBACT treatment due to its amoxicillin content. Mixed infections caused by amoxicillin - susceptible organisms in conjunction with CEROBACT-susceptible β-lactamase producing organisms may therefore be treated with CEROBACT.

[Recommended Dose]

Dosage for the treatment of infections

Adults and children Usually 1.2 g eight hourly. In more serious infections, increase frequency to six-hourly intervals.

Children 3 months-12 years: Usually 30 mg/kg * CEROBACT eight hourly. In more serious

Children 0-3 months: infections, increase frequency to six-hourly intervals.

30 mg/kg * CEROBACT every 12 hours in premature infants and in full term infants during the perinatal period, increasing to eight hours thereafter. * Each 30 mg CEROBACT contains 25 mg amoxicillin and 5 mg clavulanic acid.

Adult dosage for surgical prophylaxis

The usual dose is 1.2 g CEROBACT intravenous given at the induction of anaesthesia. Operations where there is a high risk of infection, e.g. colorectal surgery, may require three, and up to four, doses of 1.2 g CEROBACT intravenous in a 24-hour period. These doses are usually given at 0, 8, 16 (and 24) hours. This regimen can be continued for several days if the procedure has a significantly increased risk of infection.

Clear clinical signs of infection at operation will require a normal course of intravenous or oral CEROBACT therapy post-operatively.

Dosage in renal impairment

Adults

	Mild impairment (creatinine clearance >30 ml/min)	Moderate impairment (creatinine clearance 10-30 ml/min)	Severe impairment (creatinine clearance <10 ml/min)
No change in dosage	1.2 g IV of Amoxicillin & Clavulanate Injection stat*, followed by 600 mg IV CEROBACT 12 hourly	1.2 g IV of Amoxicillin & Clavulanate Injection stat*, followed by 600 mg IV CEROBACT 12 hourly	1.2 g IV of Amoxicillin & Clavulanate Injection stat*, followed by 600 mg IV CEROBACT 12 hourly. Dialysis decreases serum concentrations of CEROBACT and an additional 600 mg IV dose may need to be given during dialysis and at the end of dialysis

*Note: For stat dosing, other products of Amoxicillin & Clavulanate Injection is to be used other than CEROBACT.

Children
Similar reductions in dosage should be made for children.

Dosage in hepatic impairment
Dose with caution; monitor hepatic function at regular intervals.
Each 1.2 g vial of CEROBACT contains 1.0 mmol of potassium and 3.1 mmol of sodium (approx).

Administration
CEROBACT intravenous may be administered by intravenous intermittent infusion only. It is not suitable for intramuscular administration.

[Route of Administration]
Intravenous infusion

[Contraindications]
Hypersensitivity to the active substances, to any of the penicillins.
History of a severe immediate hypersensitivity reaction (e.g. anaphylaxis) to another beta-lactam agent (e.g. a cephalosporin, carbapenem or monobactam).
History of jaundice/hepatic impairment due to amoxicillin/clavulanic acid.

[Warning & Precautions]
Antibiotic-associated colitis has been reported with nearly all antibacterial agents including amoxicillin and may range in severity from mild to life threatening. Therefore, it is important to consider this diagnosis in patients who present with diarrhoea during or subsequent to the administration of any antibiotics. Should antibiotic-associated colitis occur, amoxicillin/clavulanic acid should immediately be discontinued, a physician be consulted and an appropriate therapy initiated. Anti-peristaltic medicinal products are contraindicated in this situation.
Periodic assessment of organ system functions, including renal, hepatic and haematopoietic function is advisable during prolonged therapy.
Prolongation of prothrombin time has been reported rarely in patients receiving amoxicillin/clavulanic acid. Appropriate monitoring should be undertaken when anticoagulants are prescribed concomitantly. Adjustments in the dose of oral anticoagulants may be necessary to maintain the desired level of anticoagulation.
In patients with renal impairment, the dose should be adjusted according to the degree of impairment.
In patients with reduced urine output crystalluria has been observed very rarely, predominantly with parenteral therapy. During the administration of high doses of amoxicillin it is advisable to maintain adequate fluid intake and urinary output in order to reduce the possibility of amoxicillin crystalluria. In patients with bladder catheters, a regular check of patency should be maintained.
During treatment with amoxicillin, enzymatic glucose oxidase methods should be used whenever testing for the presence of glucose in urine because false positive results may occur with non-enzymatic methods.
The presence of clavulanic acid in CEROBACT 1.2g may cause a non-specific binding of IgG and albumin by red cell membranes leading to a false positive Coombs test.
There have been reports of positive test results using the Bio-Rad Laboratories Platelia Aspergillus EIA test in patients receiving amoxicillin/clavulanic acid who were subsequently found to be free of Aspergillus infection. Cross-reactions with non-Aspergillus polysaccharides and polyfuranoses with Bio-Rad Laboratories Platelia Aspergillus EIA test have been reported. Therefore, positive test results in patients receiving amoxicillin/clavulanic acid should be interpreted cautiously and confirmed by other diagnostic methods.
Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients receiving therapy with beta-lactams. Before initiating therapy with CEROBACT, careful inquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenems or other beta-lactam agents. If an allergic reaction occurs, CEROBACT must be discontinued immediately and appropriate alternative therapy instituted.

[Interactions with Other Medicaments]
Oral anticoagulants
Oral anticoagulants and penicillin antibiotics have been widely used in practice without reports of interaction. However, in the literature there are cases of increased international normalised ratio in patients maintained on acenocoumarol or warfarin and prescribed a course of amoxicillin. If co-administration is necessary, the prothrombin time or international normalised ratio should be carefully monitored with the addition or withdrawal of amoxicillin. Moreover, adjustments in the dose of oral anticoagulants may be necessary.

Methotrexate
Penicillins may reduce the excretion of methotrexate causing a potential increase in toxicity.

Probenecid
Concomitant use of probenecid is not recommended. Probenecid decreases the renal tubular secretion of amoxicillin. Concomitant use of probenecid may result in increased and prolonged blood levels of amoxicillin but not of clavulanic acid.

Mycophenolate mofetil
In patients receiving mycophenolate mofetil, reduction in pre-dose concentration of the active metabolite mycophenolic acid (MPA) of approximately 50% has been reported following commencement of oral amoxicillin plus clavulanic acid. The change in pre-dose level may not accurately represent changes in overall MPA exposure. Therefore, a change in the dose of mycophenolate mofetil should not normally be necessary in the absence of clinical evidence of graft dysfunction. However, close clinical monitoring should be performed during the combination and shortly after antibiotic treatment.

[Pregnancy and Lactation]
Pregnancy
Data do not indicate direct or indirect harmful effects with respect to pregnancy, embryonal/foetal development, parturition or postnatal development. Limited data on the use of amoxicillin/clavulanic acid during pregnancy in humans do not indicate an increased risk of congenital malformations. In women with preterm, premature rupture of the foetal membrane it was reported that prophylactic treatment with amoxicillin/clavulanic acid may be associated with an increased risk of necrotising enterocolitis in neonates. Use should be avoided during pregnancy, unless considered essential by the physician.

Breastfeeding
Both substances are excreted into breast milk (nothing is known of the effects of clavulanic acid on the breast-fed infant). Consequently, diarrhoea and fungus infection of the mucous membranes are possible in the breast-fed infant, so that breast-feeding might have to be discontinued. The possibility of sensitisation should be taken into account. Amoxicillin/clavulanic acid should only be used during breast-feeding after benefit/risk assessment by the physician in charge.

[Side Effects]
The most commonly reported adverse drug reactions (ADRs) are diarrhoea, nausea and vomiting. The following terminologies have been used in order to classify the occurrence of undesirable effects. Very common, Common, Uncommon, Rare, Very rare, Not known.

Infections and infestations		Gastrointestinal disorders	
Mucocutaneous candidosis	Common	Diarrhoea	Common
Overgrowth of non-susceptible organisms	Not known	Nausea	Uncommon
		Vomiting	Uncommon
Blood and lymphatic system disorders		Indigestion	Uncommon
Reversible leucopenia (including neutropenia)	Rare	Antibiotic-associated colitis ⁴	Not known
Thrombocytopenia	Rare	Hepatobiliary disorders	
Reversible agranulocytosis	Not known	Rises in AST and/or ALT ⁵	Uncommon
Haemolytic anaemia	Not known	Hepatitis ⁶	Not known
Prolongation of bleeding time and prothrombin time ⁷	Not known	Cholestatic jaundice ⁶	Not known
Immune system disorders ⁸		Skin and subcutaneous tissue disorders ⁹	
Angioneurotic oedema	Not known	Skin rash	Uncommon
Anaphylaxis	Not known	Pruritus	Uncommon
Serum sickness-like syndrome	Not known	Urticaria	Uncommon
Hypersensitivity vasculitis	Not known	Erythema multiforme	Rare
		Drug reaction with eosinophilia and systemic symptoms (DRESS)	Very Rare
Nervous system disorders		Stevens-Johnson syndrome	Not known
Dizziness	Uncommon	Toxic epidermal necrolysis	Not known
Headache	Uncommon	Bullous exfoliative dermatitis	Not known
Convulsions ²	Not known	Acute generalised exanthematous pustulosis (AGEP) ⁹	Not known
Asceptic meningitis	Not known	Renal and urinary disorders	
Vascular disorders		Interstitial nephritis	Not known
Thrombophlebitis ³	Rare	Crystalluria ⁴	Not known

¹ See section Warning & Precautions
² See section Warning & Precautions
³ At the site of injection
⁴ Including pseudomembranous colitis and haemorrhagic colitis
⁵ A moderate rise in AST and/or ALT has been noted in patients treated with beta-lactam class antibiotics, but the significance of these findings is unknown.
⁶ These events have been noted with other penicillins and cephalosporins.
⁷ If any hypersensitivity dermatitis reaction occurs, treatment should be discontinued.
⁸ See section Symptoms and Treatment of Overdose
⁹ See section Warning & Precautions
¹⁰ See sections Contraindications and Warning & Precautions

[Symptoms and Treatment of Overdose]
Symptoms and signs of overdose
Gastrointestinal symptoms and disturbance of the fluid and electrolyte balances may be evident. Amoxicillin crystalluria, in some cases leading to renal failure, has been observed. Convulsions may occur in patients with impaired renal function or in those receiving high doses. Amoxicillin has been reported to precipitate in bladder catheters, predominantly after intravenous administration of large doses. A regular check of patency should be maintained.

Treatment of intoxication
Gastrointestinal symptoms may be treated symptomatically, with attention to the water/electrolyte balance. Amoxicillin/clavulanic acid can be removed from the circulation by haemodialysis.

[Effects on Ability to Drive and Use Machine]
No studies on the effects on the ability to drive and use machines have been performed. However, undesirable effects may occur (e.g. allergic reactions, dizziness, convulsions), which may influence the ability to drive and use machines

[Instructions for Use]
Reconstituted and diluted solution should only be used if the solution is clear and free from particles.
Preparation of solutions for reconstituted vials
1.2 g vial: To reconstitute dissolve in 10 ml of Water for Injection or the following fluids: Sodium Chloride Injection 0.9% or Sodium Lactate Ringer's Injection.
A transient pink coloration may or may not appear during reconstitution. Reconstituted solutions are normally colourless or a yellow colour.
Preparation of solutions for intravenous infusion
CEROBACT intravenous must be reconstituted as described above. Add, without delay, 1.2 g reconstituted solution to 110 ml of the same compatible fluid as used for reconstitution (e.g. using a minibag or in-line burette). Infuse over 30-40 minutes and complete within times stated in Section Shelf Life below.
Solutions should be made up to full infusion volume immediately after reconstitution. Diluted solutions for intravenous infusion are normally colourless or a slightly yellow colour.
Any residual antibiotic solutions should be discarded.
Therapy can be started parenterally and continued with an oral preparation.
Treatment should not be extended beyond 14 days without review.

[Incompatibilities]
This medicinal product must not be mixed with other medicinal products except those mentioned in section Instructions for Use
CEROBACT 1.2g Intravenous should not be mixed with blood products, other proteinaceous fluids such as protein hydrolysates or with intravenous lipid emulsions. If prescribed concomitantly with an aminoglycoside, the antibiotics should not be mixed in the syringe, intravenous fluid container or giving set because loss of activity of the aminoglycoside can occur under these conditions.
CEROBACT 1.2g solutions should not be mixed with infusions containing glucose, dextran or bicarbonate.

[Storage Condition]
Powder in vials: Store tightly closed at a temperature under 30°C and protect from light and moisture.
[Shelf Life]
Powder in vials:
24 months.
Diluted for intravenous infusion
Chemical and physical in-use stability has been demonstrated for 2-3 hours at room temperature (below 30°C), or 6-8 hours at 2-8°C. From a microbiological point of view, unless the method of opening/reconstitution/dilution precludes the risk of microbial contamination, the reconstituted and diluted solution should be used immediately. If not used immediately, in-use storage times and conditions are the responsibility of the user.
Intravenous infusions of amoxicillin/clavulanic acid may be given in a range of different intravenous fluid. Satisfactory antibiotic concentrations are retained at 2-8°C and at room temperature in the recommended volumes of the following infusion fluids. If reconstituted and maintained at room temperature, infusions should be completed within the times stated in the table below.

Intravenous infusion	Stability period at room temperature (below 30°C)
Water for injection	3 hours
Sodium Chloride Injection 0.9%	3 hours
Sodium Lactate Ringer's Injection	2 hours

#Data from laboratory test
Reconstituted solutions should not be frozen.
CEROBACT is less stable in infusions containing glucose, dextran or bicarbonate.
For storage at 2-8°C, the reconstituted solution should be added to pre-refrigerated infusion bags which may be stored for 6-8 hours. Thereafter, the infusion should be administered immediately after reaching room temperature.

Intravenous infusion	Stability period at 2-8°C
Water for injection	8 hours
Sodium Chloride Injection 0.9%	6 hours
Sodium Lactate Ringer's Injection	6 hours

#Data from laboratory test
The stability of CEROBACT solutions is concentration dependent. In the event that the use of more concentrated solutions is required, the stability period should be adjusted accordingly.

[Pack]
CEROBACT 1.2g is filled into a 12 mL Type II clear glass vial, 10 vials are packaged in each outer carton.

[Manufacturer]
 Zhuhai United Laboratories (Zhongshan) Co., Ltd.,
No.12 Jialian Road, Tanzhou Town,
Zhongshan, Guangdong, P.R.China.

[Product Registration Holder]
Pharmex Sdn Bhd, Lot 11-15, WK Commercial Centre,
Jalan Bundusan, KM 8, Penampang
88300 KOTA KINABALU SABAH, MALAYSIA

[Date of Revision] May 2022

Size: 180 x 260mm

Black

Pantone 185C

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26-May-2022

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